



Visit ID	: MPR4278559	Collected	: 08/Jul/2026 02:45PM
UHID/MR No	: APR4.0000277343	Reported	: 08/Jul/2026 04:08PM
Patient Name	: Mr.ATHARVA GUPTA	Status	: Final Report
Age/Gender	: 29 Y 0 M 0 D /M	Client Code	: 7447
Ref Doctor	: Dr.SELF	Barcode No	: E70108091
Client Name	: MODERN PATH LAB (BADSHAHPUR)	RCH ID/MCTSID	:

DEPARTMENT OF HAEMATOLOGY

SWASTHYA PANEL 70

Test Name	Result	Unit	Bio. Ref. Range	Method
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CBC

Sample Type : WHOLE BLOOD EDTA

HAEMOGLOBIN (HB)	16.4	gm/dl	13.00-17.00	Photometry
PCV/HAEMATOCRIT	49.3	%	40-50	Calculated
MCV	114.3	fL	80-100	Electrical Impedence
MCH	38	pg	27-32	Calculated
MCHC	33.3	g/dl	32-36	Calculated
PLATELET COUNT	2.15	lacs/cmm	1.50 - 4.50	Electrical Impedence
RBC COUNT(RED BLOOD CELL COUNT)	4.31	million/Cells/uL	4.50-5.50	Electrical Impedance
TOTAL LEUCOCYTE COUNT (TLC)	7,800	thousand/uL	4000-10000	Electrical Impedence
DLC (by Automated/ Microscopy)				
NEUTROPHIL	58.00	%	37-70	
LYMPHOCYTE	30.00	%	22 - 55	
EOSINOPHIL	6.00	%	01-07	
MONOCYTE	6.00	%	2-10	
BASOPHIL	0.00	%	00-02	
RDW-CV	14.5	%	12.0 - 14.1	Automated/Calculated
RDW-SD	57.3	fL	35.3 - 41.0	Calculated
MPV	8.1	fL	8.6 - 11.5	Electrical Impedence
ABSOLUTE NEUTROPHIL COUNT	4.60	x10 ³ Cells/uL	1.5-7.8	Automated Calculated
ABSOLUTE LYMPHOCYTE COUNT	2.30	x10 ³ Cells/uL	2.0-3.9	Automated Calculated
ABSOLUTE EOSINOPHIL COUNT	0.50	x10 ³ Cells/uL	0.02 - 0.50	Flow Cytometry
ABSOLUTE MONOCYTE COUNT	0.40	x10 ³ Cells/uL	0.20 - 1.00	Flow cytometry
ABSOLUTE BASOPHIL COUNT	0.00	x10 ³ Cells/uL	0 - 1	Automated Calculated





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HBA1C

Sample Type : WHOLE BLOOD EDTA

HBA1c	5.2	%	Normal Glucose tolerance (non-diabetic): <5.7%~Pre-diabetic: 5.7-6.4%~Diabetic Mellitus: >6.5%	HPLC
ESTIMATED AVG. GLUCOSE	101.39	mg/dl		

COMMENTS

HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations.

INTERPRETATION :

As per American Diabetes Association (ADA)

Reference Group	Hba1c in %
Non diabetic adults >=18 years	< 5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Action suggested: > 8.0 Age < 19 Years - Goal of therapy : < 7.5

INCREASED IN

1. Chronic renal failure with or without hemodialysis.
2. Iron deficiency anemia. Increased serum triglycerides.
3. Alcohol.
4. Salicylate treatment.

DECREASED IN

1. Shortened RBC life span (hemolytic anemia, blood loss), Pregnancy.
2. Ingestion of large amounts (>1g/day) of vitamin C or E.
3. Hemoglobinopathies (e.g.: spherocytes) produce variable increase or decrease.
4. Results of %HbA1c are not reliable in patients with chronic blood loss and consequent variable erythrocyte life span.

Note:

1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled.
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.





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Age/Gender : 29 Y 0 M 0 D /M	Client Code : 7447
Ref Doctor : Dr.SELF	Barcode No : S70108091
Client Name : MODERN PATH LAB (BADSHAHPUR)	RCH ID/MCTSID :

DEPARTMENT OF BIOCHEMISTRY

SWASTHYA PANEL 70

Test Name	Result	Unit	Bio. Ref. Range	Method
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LIVER FUNCTION TEST

Sample Type : SERUM

TOTAL BILIRUBIN	0.90	mg/dl	0.3-1.2	Diazonium salt(Colorimetric)/Jendressik
CONJUGATED (D. Bilirubin)	0.29	mg/dl	0.00-0.30	Vanadate Oxidation
UNCONJUGATED (I.D. Bilirubin)	0.61	mg/dl	0.10-1.00	Calculated
TOTAL PROTEINS	7.80	g/dl	5.7-8.2 g/dl	Biuret
ALBUMIN	5.06	gm/dl	3.5-5.0	BCG
GLOBULIN	2.74	gm/dL	2.0-4.1	Calculated
A/G RATIO	1.85		1.0-2.0	Calculated
SGOT	24.00	U/L	<34	Modified IFCC
SGPT	26.00	U/L	10.0-49.0	Enzymatic,IFCC
GGT	26.00	U/L	<73 U/L	Modified IFCC
ALKALINE PHOSPHATASE	84.00	U/L	30-120	IFCC

LIPID PROFILE

Sample Type : SERUM

TOTAL CHOLESTEROL	132	mg/dl	Low-risk levels (desirable)~<200~Moderate-risk levels~(borderline) 200-239~High-risk levels ≥ 240	Cholesterol oxidase/peroxidase
H D L CHOLESTEROL	30.60	mg/dl	40-60	Phosphotungstate/Mg-Cholesterol oxidase/peroxidase
L D L CHOLESTEROL	58.6	mg/dl	70-106~Above Optimal : 100-129~Borderline High : 130-159~High : 160-189~Very High : ≥190	Absorption Spectrometry [Peroxidase]
TRIGLYCERIDES	214	mg/dl	Adult Serum ~Normal:<150 ~BorderLine hight:150-199 ~High : 200-499 ~Very High:≥500	GPO
VLDL	42.80	mg/dl	15-30	Calculated
NON HDL CHOLESTEROL	101.4	mg/dl	Desirable: <130~BorderLine : 150-199~High : 200-499~Very High : ≥500	Calculated
T. CHOLESTEROL/ HDL RATIO	4.31			Calculated
LDL / HDL RATIO	1.92			Calculated





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DEPARTMENT OF BIOCHEMISTRY**SWASTHYA PANEL 70**

Test Name	Result	Unit	Bio. Ref. Range	Method
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TOTAL CALCIUM**Sample Type : SERUM**

SERUM TOTAL CALCIUM	8.70	mg/dl	8.3-10.6	cpc
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INTERPRETATION:

-Calcium level is increased in patients with hyperparathyroidism, Vitamin D intoxication, metastatic bone tumor, milk-alkali syndrome, multiple myeloma, Paget's disease.
-Calcium level is decreased in patients with hemodialysis, hypoparathyroidism (primary, secondary), vitamin D deficiency, acute pancreatitis, diabetic Keto-acidosis, sepsis, acute myocardial infarction (AMI), malabsorption, osteomalacia, renal failure, rickets.

PLASMA GLUCOSE - FASTING**Sample Type : FLOURIDE PLASMA**

Plasma Glucose Fasting	89	mg/dl	60-110	Glucose Oxidase/Peroxidase
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INTERPRETATION:**Increased In**

- Diabetes Mellitus
- Stress (e.g., emotion, burns, shock, anesthesia)
- Acute pancreatitis
- Chronic pancreatitis
- Wernicke encephalopathy (vitamin B1 deficiency)
- Effect of drugs (e.g. corticosteroids, estrogens, alcohol, phenytoin, thiazides)

Decreased In

- Pancreatic disorders
- Extrapaneatronic tumors
- Endocrine disorders
- Malnutrition
- Hypothalamic lesions
- Alcoholism
- Endocrine disorders

Note : Test result can vary as per the timing of sample collection, duration of fasting, timing between collection and testing, timing of meal and type of diet taken, medication, physical activity before collection of sample etc. Test results needs to be correlated clinically.





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IRON PROFILE -I**Sample Type : SERUM**

SERUM IRON	172.00	ug/dl	65-175	Ferrozine
TOTAL IRON BINDING CAPACITY	280	ugm/dl	250-425	spectrophotometer
UIBC	108.00	ugm/dL	130 - 336	Calculated
TRANSFERRIN SATURATION	61.43	%	16-50	

INTERPRETATION:**SERUM IRON INCREASED IN:**

- Hemosiderosis of excessive iron intake (e.g. repeated blood transfusion, iron therapy, iron containing vitamins).
- Decreased formation of RBCs (thalassemia, pyridoxal deficiency anaemia).
- Increased destruction of RBCs (hemolytic anaemia).
- Acute liver damage
- Acute iron toxicity

SERUM IRON DECREASED IN:

- Iron deficiency anaemia
- Normochromic anaemia of infections & chronic diseases
- Nephrosis
- Menorrhagia
- Diurnal variation: Normal in mid morning, low values in mid afternoon, and very low values near midnight.

TIBC/UIBC INCREASED IN:

- Iron deficiency anemia
- Acute & Chronic blood loss
- Acute liver damage
- Progesterone birth control pills

TIBC/UIBC DECREASED IN:

- Hemochromatosis
- Cirrhosis of the liver
- Thalassemia
- Anemia of infective & chronic disease
- Nephrosis

TRANSFERRIN SATURATION INCREASED IN:

- High Values in iron overload
- Raised transferrin saturation is an early indicator of Iron accumulation in hemochromatosis.

TRANSFERRIN SATURATION DECREASED IN:

- Low Values in iron deficiency





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SWASTHYA PANEL 70

Test Name	Result	Unit	Bio. Ref. Range	Method
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KIDNEY FUNCTION TEST

Sample Type : SERUM

SERUM UREA	13.33	mg/dL	19-49	Urease GLDH
TOTAL PROTEINS	7.80	g/dl	5.7-8.2 g/dl	Biuret
ALBUMIN	5.06	gm/dl	3.5-5.0	BCG
GLOBULIN	2.74	gm/dL	2.0-4.1	Calculated
A/G RATIO	1.85		1.0-2.0	Calculated
BUN	6.23			
SERUM CREATININE	0.7	mg/dl	0.55-1.3	Jaffe, alkaline picrate, kinetic with blank rate correction
BUN / CREATININE RATIO	9			
SERUM SODIUM	138.91	mmol/L	136 - 146	ISE - Indirect
SERUM POTASSIUM	4.76	nmol/L	3.5 - 5.1	ISE - Indirect
SERUM CHLORIDE	104.72	mmol/L	98-109	ISE - Indirect
SERUM URIC ACID	4.40	mg/dL	3.7 - 9.2	Uricase/peroxidase
SERUM TOTAL CALCIUM	8.70	mg/dl	8.3-10.6	cpc
Estimated Glomerular Filtration Rate (eGFR)	128.00	mL/min/1.73m ²		

INTERPRETATION:

- Estimated GFR (eGFR) calculated using the 2021 CKD-EPI creatinine equation.
- CKD-EPI 2021 equation is most accurate for GFR ≤ 60 mL/min/1.73m².
- The BUN-to-creatinine ratio is used to differentiate prerenal and post renal azotaemia from renalazotemia. Because of considerable variability, it should be used only as a rough guide. Normally, the BUN/creatinine ratio is about 10:1.

CKD stage	Description	GFR	Associated findings
0	Normal kidney function	>90	No proteinuria
1	Kidney damage with normal or high GFR	>90	Presence of Protein, albumin, cells or casts in urine
2	Kidney damage with mild loss of kidney function	60-89	-
3a	Mild to moderate loss of kidney function	45-59	-
3b	Moderate to severe loss of kidney function	30-44	-
4	Severe loss of kidney function	15-29	-
5	Kidney Failure	<15	-

References : National Kidney Foundation <https://www.kidney.org>





Visit ID	: MPR4278559	Collected	: 08/Jul/2026 02:45PM
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DEPARTMENT OF HORMONE ASSAYS

Test Name	Result	Unit	Bio. Ref. Range	Method
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PROLACTIN

Sample Type : SERUM

PROLACTIN	7.39	ng/ml	REFER INTERPRETATION	CLIA
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INTERPRETATION:

Normal Female	2.41-27.36 ng/ml
Post Menopause	1.7-21.3 ng/ml
Gestation Period	10-210 ng/ml
Male	2-19.49 ng/ml

-The major physiologic action of prolactin is the initiation and maintenance of lactation in women.
 -Hyperprolactinemia has been established as a common cause of infertility and gonadal disorders in men and women.
 -Causes of increased prolactin concentrations include pituitary tumours, amenorrhoea and/or galactorrhoea, primary hypothyroidism, anorexia nervosa, polycystic ovarian syndrome, renal failure and ectopic production. Women taking oral contraceptives or receiving estrogen therapy can have elevated prolactin concentration. Stress, coitus, some psychotropic and antihypertensive drugs may give falsely elevated values.
 -Causes of decreased prolactin concentrations include hypopituitarism, post partum, administration of certain drugs like L-dopa, apomorphine, clonidine and bromocriptine.

LIMITATIONS:

-Prolactin levels have been found to be influenced by various factors other than the diseased state.
 -Prolactin may exist in alternate structural forms (e.g. macroprolactin) which may exhibit variable levels of physiological activity. Additional information may be required for diagnosis.
 -Specimens from patients who have received preparations of mouse monoclonal antibodies for diagnosis or therapy may show either falsely elevated or depressed values.





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DEPARTMENT OF HORMONE ASSAYS

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FERRITIN**Sample Type : SERUM**

FERRITIN	123.3	ng/mL	21.81-274.66	CLIA
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INTERPRETATION:

Ferritin is a high-molecular weight iron containing protein that functions in the body as an iron Storage compound. Ferritin provides a more sensitive, specific and reliable measurement for determining iron deficiency at an early stage. The combined use of serum ferritin levels and mean corpuscular volume (MCV) has made differentiation between iron deficiency, beta-thalassemia trait and normal subjects possible at a very high level of accuracy. Serum ferritin measurements provide important clinical parameters for assessing the response to treatment with deferoxamine, in the treatment of thalassemia. Elevated levels are seen in malignant diseases such as leukemia, Hodgkins disease, breast cancer, head and neck cancer and ovarian cancer.

LIMITATIONS:

Specimens from patients who have received preparations of mouse monoclonal antibodies for diagnosis or therapy may show either false positive or depressed values.

For diagnostic purposes the ferritin result should be used in conjunction with other data, e.g.: symptoms, results of other tests, clinical impressions, etc.





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Patient Name	: Mr.ATHARVA GUPTA	Status	: Final Report
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DEPARTMENT OF HORMONE ASSAYS

Test Name	Result	Unit	Bio. Ref. Range	Method
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TESTOSTERONE TOTAL**Sample Type : SERUM**

TESTOSTERONE TOTAL	480.08	ng/dl	164.94-753.38	CLIA
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CLINICAL USE:

Measurement of testicular function in males.
Management of hirsutism and virilization in females.

INCREASED LEVEL:

- Precocious puberty in males
- Androgen resistance
- Testotoxicosis
- Congenital adrenal hyperplasia
- Polycystic ovarian disease
- Ovarian tumour

DECREASED LEVEL:

- Delayed puberty in males
- Gonadotropin deficiency
- Testicular defects





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SWASTHYA PANEL 70

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25 HYDROXY VITAMIN D

Sample Type : SERUM

VITAMIN D	17.45	ng/ml	30-100	CLIA
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INTERPRETATION:

LEVEL	REFERENCE RANGE (Adult)	REFERENCE RANGE (Pediatric)
Deficiency	< 20 ng/ml	<15 ng/ml
Insufficiency	20-30 ng/ml	15-20 ng/ml
Sufficiency	30-100 ng/ml	20-100 ng/ml
Toxicity	> 100 ng/ml	> 100 ng/ml

DECREASED LEVELS:

- Deficiency in children causes Rickets and in adults leads to Osteomalacia. It can also lead to Hypocalcemia and Tetany.
- Inadequate exposure to sunlight.
- Dietary deficiency.
- Vitamin D malabsorption.
- Severe Hepatocellular disease.
- Drugs like Anticonvulsants.
- Nephrotic syndrome.

INCREASED LEVELS:

- Vitamin D intoxication.

COMMENTS:

- Vitamin D (Cholecalciferol) promotes absorption of calcium and phosphorus and mineralization of bones and teeth. Vitamin D status is best determined by measurement of 25 hydroxy vitamin D, as it is the major circulating form and has longer half life (2-3 weeks) than 1, 25 Dihydroxy vitamin D (5-8 hrs).
- The assay measures D3 (Cholecalciferol) metabolites of vitamin D.
- 25 (OH) D is influenced by sunlight, latitude, skin pigmentation, sunscreen use and hepatic function.
- Optimal calcium absorption requires vitamin D 25 (OH) levels exceeding 75 nmol/L.
- It shows seasonal variation, with values being 40-50% lower in winter than in summer.
- Levels vary with age and are increased in pregnancy.
- This is the recommended test for evaluation of vitamin D intoxication.





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DEPARTMENT OF HORMONE ASSAYS**SWASTHYA PANEL 70**

Test Name	Result	Unit	Bio. Ref. Range	Method
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VITAMIN B12**Sample Type : SERUM**

VITAMIN B12	271.00	pg/mL	211-911 pg/mL	CLIA
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COMMENTS:

Results may differ between laboratories due to variation in population and test method. Vitamin B12 is implicated in the formation of myelin, and along with Folate is required for DNA synthesis. The most prominent source of B12 for humans is meat while untreated fresh water can also be a source.

Megaloblastic anaemia has been found to be due to B12 deficiency, a major cause being Pernicious anemia due to poor B12 uptake resulting in below normal serum levels. Other conditions related to low B12 levels include iron deficiency anemia, pregnancy, vegetarianism, partial gastrectomy, ileal damage, oral contraceptives, parasitic infestations, pancreatic deficiency, treated epilepsy and advancing age. The correlation of serum B12 levels and Megaloblastic anemia however is not always clear - some patients with high MCV may have normal B12 levels, while some individuals with B12 deficiency may not have megaloblastic anemia. Disorders renal failure, liver diseases and myeloproliferative diseases may have elevated vitamin B12 levels.

LIMITATIONS:

For diagnostic purposes, the B12 results should be used in conjunction with other data; e.g.; symptoms results of other testing, clinical impressions, etc.

If the B12 level is inconsistent with clinical evidence, additional testing is suggested to confirm the result.





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THYROID PROFILE (T3,T4,ULTRASENSITIVE TSH)

Sample Type : SERUM

T3	1.27	ng/mL	0.60-1.81	CLIA
T4	11.60	ug/dl	3.2-12.6	CLIA
Ultrasensitive TSH	1.139	uIU/mL	0.55-4.78	CLIA

INTERPRETATION:

1. Serum T3, T4 and TSH are the measurements form three components of thyroid screening panel and are useful in diagnosing various disorders of thyroid gland function.
2. Primary hyperthyroidism is accompanied by elevated serum T3 and T4 values along with depressed TSH levels.
3. Primary hypothyroidism is accompanied by depressed serum T3 and T4 values and elevated serum TSH levels.
4. Normal T4 levels accompanied by high T3 levels are seen in patients with T3 thyrotoxicosis. Slightly elevated T3 levels may be found in pregnancy and in estrogen therapy while depressed levels may be encountered in severe illness, malnutrition, renal failure and during therapy with drugs like propranolol and propylthiouracil.
5. Although elevated TSH levels are nearly always indicative of primary hypothyroidism, rarely they can result from TSH secreting pituitary tumors (secondary hyperthyroidism).
6. Low levels of Thyroid hormones (T3, T4 & FT3, FT4) are seen in cases of primary, secondary and tertiary hypothyroidism and sometimes in non-thyroidal illness also.
7. Increased levels are found in Grave's disease, hyperthyroidism and thyroid hormone resistance.
8. TSH levels are raised in primary hypothyroidism and are low in hyperthyroidism and secondary hypothyroidism.

9. REFERENCE RANGE:

PREGNANCY	Ultrasensitive TSH in uIU/mL
1st Trimester	0.100 – 2.500
2nd Trimester	0.200 – 3.000
3rd Trimester	0.300 – 3.000

(Reference range recommended by the American Thyroid Association)

Comments :

1. During pregnancy, Free thyroid profile (FT3, FT4 & Ultra-TSH) is recommended.TSH levels are subject to circadian variation, reaches peak levels between 2-4 AM and at a minimum between 6-10 PM. The variation of the day has influence on the measured serum TSH concentrations.

Limitations

- Patient samples may contain heterophilic antibodies that could react in immunoassays to give falsely elevated or depressed results
- Do not use samples that contain fluorescein. Fluorescein levels > 0.24 µg/mL may decrease results in this assay. . Evidence suggests that patients undergoing retinal fluorescein angiography can retain amounts of fluorescein in the body for up to 48–72 hours post-treatment
- As with any immuno-recognition measurement of a peptide, extremely rare genetic variants may exhibit varying degrees of detection

*** End Of Report ***

Anjali Kwatra

Dr. Anjali Kwatra
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Kanika

Dr. Kanika Yadav
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Consultant Pathologist
Reg. No. - 011681



HBa1c Reports

Sample Id : E70108091

Sample Processed Date: 08/07/202026 / 15:26

Peak Name	Retention Time(s)	Absorbance	Area	Result (Area %)
Hba1a	11.2	32	1005	0.55
Hba1b	13.7	54	1389	0.77
HbF	20.1	21	934	0.51
La1c	25.5	51	2783	1.54
HbA1c	38.6	76	6480	5.16
UnKnown	62.5	3.42	6165	3.42
HbA0	69.0	5283	161304	89.58

HbA1c(NGSP)= 5.16

TotalArea= 180060.0

